A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

25 May 2026

Dr Amanda Richards
Pinnacle Surgery

Dear Dr Richards,

Re: Ms Keryn Le Page
DOB: 22/09/1967 Phone: 0478 408 840
Address: 12 Cusdin Street, GLEN IRIS, VIC, 3146
Email: keri.lepage@gmail.com

Summary

- **Subglottic stenosis with potential silent reflux or bile reflux as a contributor.**
- **Dysphagia symptoms, specifically with pills getting stuck in the mid-throat region.**
- **For gastroscopy and biopsies to exclude structural reflux, eosinophilic oesophagitis, and other structural pathology.**
- **For an expanded autoimmune blood screen to investigate potential systemic contributors to subglottic stenosis.**
- **Trial of lifestyle modifications including earlier dinner times and avoiding snacks before bed.**
- **No action required by you.**

I had the pleasure of conducting an appointment with Keryn Le Page. Thank you for referring her.

Symptoms and history of presenting complaint

As you are aware, Keryn has a history of subglottic stenosis and has undergone approximately six procedures to date. We discussed whether gastro-oesophageal reflux may be contributing to her condition, particularly as her treatment frequency is higher than typical. Keryn does not experience classical symptoms of heartburn or regurgitation. However, she does describe a "lump" feeling in her throat and breathing difficulties that she feels have evolved over the last few years.

She reports specific swallowing difficulties, particularly a sensation of pills getting stuck in the mid-to-lower throat area. This occurs relatively frequently with medication, though she notes that food sticking is a rare occurrence. Interestingly, she noted that her respiratory symptoms seemed to worsen following her gallbladder surgery approximately two to three years ago.

Diet

Keryn maintains a generally healthy diet. She does not consume dairy. Her caffeine intake is limited to green tea, and she does not consume coffee, soft drinks, or energy drinks. She consumes wheat-based products including bread, pasta, and rice. She does report eating dinner relatively late and occasionally having tea or snacks between dinner and bedtime. Her alcohol consumption appears to be modest.

Past medical history and medications

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Keryn's medical history is significant for subglottic stenosis and a cholecystectomy performed a few years ago. She has no history of hypertension, high cholesterol, or diabetes.

Her current medications consist entirely of supplements, including Vitamin D3, zinc carnosine, "GI Revive", and Floramyces (Saccharomyces). She is not currently taking any proton pump inhibitors or other anti-reflux medications.

Treatments trialled before and investigations performed

Keryn has undergone multiple endoscopic interventions for her subglottic stenosis.

Recent blood tests performed via Melbourne Pathology show normal renal function, liver function, and fasting glucose. Her Vitamin D, B12, and folate levels are sufficient, and her iron studies and full blood examination were unremarkable. An ANA was performed and was negative, but a more comprehensive autoimmune screen has not yet been completed.

Social history, family history

Keryn works for MARS in a corporate capacity. She lives with her two children, aged 23 and 16. She grew up in New Zealand, with family origins in Christchurch region, which I noted has a high prevalence of autoimmune conditions. Her family history is significant for thyroid disease in her brother and a grandfather with some oesophageal pathology (she described it as a oesophageal narrowing). She denies a family history of asthma, eczema, or inflammatory bowel disease.

Examination

This was a telehealth consultation; however, Keryn reported an episode of visible haematuria in the past. Her recent pathology results were reviewed during the call, showing a slightly elevated cholesterol marker but otherwise stable biochemistry.

Impression and plan

While Keryn lacks the classic symptoms of gastro-oesophageal reflux, it remains possible that "silent" reflux or bile reflux (post-cholecystectomy) is contributing to the inflammation of her subglottic stenosis.

I have proposed a gastroscopy to investigate her dysphagia and to look for signs of reflux or eosinophilic oesophagitis, which can occasionally present with vocal cord-related symptoms. Given her subglottic stenosis, this will be arranged in a hospital setting rather than a standalone day centre to ensure appropriate anaesthetic support is available, although we will not be intubating her for the procedure.

I have also organised further blood tests to screen for rarer autoimmune conditions that can be associated with subglottic stenosis, such as granulomatosis with polyangiitis (GPA), especially in light of her history of haematuria. From a lifestyle perspective, I have suggested she move her dinner to an earlier time and avoid any intake (including tea) between dinner and sleep to minimise potential nocturnal reflux.

Thank you again for referring this patient.

Yours sincerely,

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A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Sana Al-Rubaye

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